OPERATION: Direct suspension microlaryngoscopy with microscopic surgical excision of left true vocal cord polyp using cold instruments.

INDICATION: A 44-year-old vocal coach with chronic hoarseness (dysphonia), vocal fatigue, and videostroboscopy demonstrating a gelatinous, pedunculated polyp on the free edge of the left true vocal cord.

SURGICAL TECHNIQUE: The patient was placed in the supine position under general endotracheal anesthesia using a 5.5 mm microlaryngeal endotracheal tube (MLT). The head was positioned in the 'sniffing' position with tooth protection in place. A Dedo laryngoscope was introduced into the oral cavity, passing over the tongue base to elevate the epiglottis. The laryngoscope was suspended using a Boston suspension gallows, providing panoramic exposure of the anterior commissure, true vocal cords, false vocal cords, and subglottic space.

The operating microscope equipped with a 400 mm focal lens was positioned. Microscopic examination revealed a 4 mm translucent, gelatinous, pedunculated polyp arising from the junction of the anterior one-third and posterior two-thirds of the left true vocal cord (striking zone). The contralateral right vocal cord revealed mild contact reactive thickening without ulceration.

Using delicate microlaryngeal instruments, the polyp was elevated medially with micro-cup forceps. The superficial lamina propria was identified and preserved. Utilizing curved micro-scissors, the stalk of the polyp was transected precisely flush with the free vibratory margin of the true vocal cord, avoiding deeper ligamentous injury. The mucosal edges reapproximated smoothly. Hemostasis was spontaneous without requiring cautery. The suspension apparatus was released, and the patient extubated smoothly. Strict total voice rest was prescribed for five days.